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Case Name: *Regional Medical Center of San Jose et al. v. County of Santa Clara dba Valley Health Plan*

Case No.: 20CV374597

I. Introduction and Summary of Rulings

In this quantum meruit action, plaintiffs Good Samaritan Hospital, L.P. and San Jose Healthcare System, L.P. (the “Hospitals”) seek the reasonable value of emergency and post-stabilization services rendered to members of the health plan operated by defendant County of Santa Clara, doing business as Valley Health Plan (the “County”). Approximately 13,189 claims are at issue; the sampling frame from which the Hospitals’ expert drew comprised 13,188. (Reiter Decl., Ex. E, ¶ 29 & fn. 12.) Three motions are before the Court, all brought by the County.

The first seeks to strike the opinions of Peter Dressel and limit those of Dr. Gregory Palega under Code of Civil Procedure section 2034.300, or, alternatively, leave to augment the County’s expert disclosure to add Dr. Paul Diver and Dr. David Schriger under sections 2034.610 and 2034.620. (County’s Memorandum of Points and Authorities in support of its Motion to Strike (“Mot.”), pp. 1-2, 8-12.) The second and third are motions to seal.

1. The motion to strike is GRANTED IN PART and DENIED IN PART. Mr. Dressel’s Opinion No. 1, directed at the County’s adjudication rules and payment framework, is stricken. The motion is otherwise denied as to Mr. Dressel, without prejudice to a timely motion in limine or motion under *Sargon Enterprises, Inc. v. University of Southern California* (2012) 55 Cal.4th 747.
2. The motion to limit Dr. Palega’s opinions is GRANTED as modified. No party or expert may offer opinion testimony extrapolating the results of Dr. Palega’s review to claims he did not review.
3. The alternative motion to augment is GRANTED IN PART, subject to the scope limitations and conditions in Part III.E.
4. The July 10, 2026 motion to seal is GRANTED IN PART and DENIED IN PART WITHOUT PREJUDICE.

5. The August 12, 2026 motion to seal is DENIED.

II. Background

The parties exchanged expert disclosures under section 2034.260 on September 3, 2025, and supplemental disclosures under section 2034.280 on December 12, 2025. Dr. Palega was disclosed as having “expertise in auditing medical records to determine if emergency services were medically necessary,” and as expected to opine on whether services rendered to certain plan members “were medically necessary emergency care.” (Reiter Decl., Ex. A at p. 5, ¶¶ 6-7.) Mr. Dressel was disclosed as having “expertise and knowledge concerning data processing and analysis of voluminous records and medical files,” and as expected to opine “regarding whether the Hospitals’ claims coding and billing practices are reasonable and appropriate”—a description repeated verbatim in December. (*Id.*, Ex. A at p. 6, ¶¶ 12-13; Ex. B at p. 7.) The County designated three experts, all on reimbursement rates, and no physician. (*Id.*, Ex. I, J.)

Meanwhile, the Hospitals sought identification of the claims to which the County contended a claim-specific defense applied. (Forcini Decl., Ex. 1; Hospitals’ Opposition to Motion to Strike (“Opp.”) at pp. 7:21-8:15.) On December 30, 2025, the Court ordered code-compliant responses to Special Interrogatories Nos. 14 through 26 without objection except for privilege or protective order. (Dec. 30, 2025 Order at pp. 8:16-19.) The County’s supplemental responses identified “[e]very claim at issue in this litigation.” (Mar. 25, 2026 Minute Order re Sanctions at p. 2.) The Court denied the Hospitals’ ensuing sanctions motion for want of a demonstrated violation but added that the County’s “responses appear to the Court to skirt the intent of the order as it may be reasonably inferred,” and that the issue “can further [be] addressed, if necessary, at the time of trial.” (*Id.* at p. 3.) The denial was “without prejudice at this time.” (*Id.* at p. 4.)

Opening expert reports were exchanged on March 5, 2026. Mr. Dressel’s report disclosed for the first time that he had been asked to “design a statistically valid sampling methodology for purposes of analyzing the claims at issue,” and had used stratified random sampling to select 180 claims; it offers two opinions, addressed separately below. (Reiter Decl.,

Ex. E, ¶¶ 9, 18, 29-35.) Dr. Palega reviewed the medical records for 178 of the 180 sampled claims and opined on whether each patient received emergency services and care, when each was stabilized, and whether post-stabilization care was medically necessary. (*Id.* at Ex. F, ¶¶ 19-20.) Later that month, on the County’s ex parte application following the withdrawal of its trial counsel, the Court vacated the May 4, 2026 trial date and remaining pretrial deadlines, advising that the continuance would “not affect the prior discovery deadlines that have already passed. You don’t get a redo,” but that “the only exception to the otherwise close of discovery are those pending outstanding dates for expert rebuttal reports and expert discovery. Both shall remain open.” (Forcini Decl., Ex. 12 at p. 32:8-16.)

Present counsel appeared April 27, 2026, and trial was reset to January 19, 2027. The parties’ June 29, 2026 stipulated order set rebuttal reports for July 31, 2026, an expert discovery cutoff of September 18, 2026, and a December 1, 2026 deadline for motions in limine and Sargon motions. (June 29, 2026 Stipulated Order, ¶¶ 1-2, 4, 8.) The County sought the Hospitals’ stipulation to add Drs. Diver and Schriger on June 30; the Hospitals declined on July 9; and this motion followed the next day. (Reiter Decl., ¶¶ 7-8 & Ex. C, G-H.)

III. Motion to Strike or, in the Alternative, to Augment

A. Legal Standard

Section 2034.260, subdivision (c) requires two distinct statements: a “brief narrative statement of the qualifications of each expert,” and a “brief narrative statement of the general substance of the testimony that the expert is expected to give.” (Subds. (c)(1), (c)(2); County’s Reply in support of its Motion to Strike (“Reply”), p. 4:11-15.) On objection by a party who has itself complied, the court “shall exclude from evidence the expert opinion of any witness” whose proponent “has unreasonably failed to” comply. (Code Civ. Proc., § 2034.300.) That reaches not only an omitted declaration but one that “fails to disclose the general substance of the testimony the party later wishes to elicit from the expert at trial”; a party wishing to go beyond the declaration “must successfully move for leave to amend.” (*Bonds v. Roy* (1999) 20 Cal.4th 140, 148-149; Mot., p. 9:6-8; Reply, pp. 1:19-23, 8:12-15.)

Disclosure is measured for generality, not particularity: testimony is adequately disclosed if within the “general ambit” of the declaration. (*Jones v. Moore* (2000) 80 Cal.App.4th 557, 566; Opp., pp. 12:24-13:7.) Exclusion also requires two findings, not one: noncompliance, and that the failure was *unreasonable*; the second finding turns on whether the party’s conduct “gives the appearance of gamesmanship” and compromises the purposes of the Discovery Act, including safeguarding against surprise. (*Du-All Safety, LLC v. Superior Court* (2019) 34 Cal.App.5th 485, 499; Opp., p. 14:11-16.)

B. Mr. Dressel’s Opinion No. 1 Is Stricken

The Hospitals disclosed that Mr. Dressel would opine “regarding whether the Hospitals’ claims coding and billing practices are reasonable and appropriate,” and repeated that description verbatim in December. (Reiter Decl., Ex. A at p. 6, ¶ 13; *Id.*, Ex. B at p. 7.) Opinion No. 1 is not that opinion. It is an opinion that the County’s “unilateral application of its internal adjudication rules and Medicare-based payment framework to out-of-network hospitals is inconsistent with established managed care industry customs.” (*Id.*, Ex. E, ¶ 18(a).) The disclosed subject is the conduct of the Hospitals; the opinion offered is directed at the conduct of the County. They are not the same subject matter, and the second is not within the “general ambit” of the first. The Hospitals do not contend otherwise; their argument addresses only Dr. Palega’s disclosure and whether the sample needed to be disclosed, and never mentions Opinion No. 1. (Opp., pp. 12:14-15:7.)

Nor is the deficiency cured by the reservation that Mr. Dressel “may testify on all other issues within his expertise that may be raised by the parties or their witnesses.” (Reiter Decl., Ex. A at p. 6, ¶ 14; relied on at Opp., p. 14:1-7.) That language appears verbatim in each of the Hospitals’ expert paragraphs. (*Id.* at ¶¶ 5, 8, 11, 14.) A reservation permitting testimony on any subject within an expert’s expertise discloses the general substance of nothing, and so reading subdivision (c)(2) would deprive it of content. (Reply, p. 4:15-20.)

The Court further finds the failure unreasonable. The Hospitals had a second opportunity at the December 12, 2025 exchange and instead repeated the earlier description word for word, so the County had no occasion to consider retaining a responsive expert on a

subject it was never told would be addressed. (Reiter Decl., Ex. B at p. 7.) Opinion No. 1, and the supporting discussion at paragraphs 19 through 28 of the report, are stricken. Mr. Dressel may not offer opinion testimony concerning the County’s adjudication rules, its payment methodology, or their consistency with managed care industry custom.

C. Mr. Dressel’s Opinion No. 2 Is Not Stricken

Opinion No. 2—that stratified random sampling provides a valid framework for auditing the disputed claims—presents a closer question, and the Court declines to strike it.

The County’s argument on the first prong has merit. The Hospitals rely on the description of Mr. Dressel’s expertise in “data processing and analysis of voluminous records and medical files,” arguing that his creation of a 180-claim sample falls within the “general ambit” of that disclosure. (Opp., p. 14:1-7.) But that language appears in the qualifications paragraph, which satisfies subdivision (c)(1); it does not discharge the separate obligation under subdivision (c)(2). Neither the Hospitals nor the Court identifies any opinion in the report addressing the reasonableness of the Hospitals’ own coding and billing practices, the only subject disclosed for him. (Reiter Decl., Ex. E, ¶ 18.)

The motion nonetheless fails on the second prong, for three reasons.

First, the sampling did not arise in a vacuum. The County resisted the Hospitals’ efforts to learn which claims it actually disputed, and after this Court ordered code-compliant responses it answered by identifying every claim in the litigation—responses this Court has already observed “appear . . . to skirt the intent of the order as it may be reasonably inferred.” (Mar. 25, 2026 Minute Order re Sanctions, pp. 2-3.) A party that reviews a sample in those circumstances is not engaged in the gamesmanship section 2034.300 addresses. (See Opp., pp. 14:26-15:7.)

Second, the posture was symmetrical: the Hospitals’ own verified interrogatory responses offered to meet and confer to narrow the County’s requests “to a reasonable subset of claims for which VHP identifies this issue.” (Suppl. Reiter Decl., Ex. 3 at p. 29:7-11; Reply, pp. 2:8-10, 5:3-4.) Each side asked the other to identify the disputed subset first, and neither position supplies a basis for mandatory exclusion.

Third, any surprise has been cured, and the harm the County identifies is met by a narrower order. The County has had the report since March 5, 2026, expert discovery runs to September 18, 2026, and the County has served two responsive reports addressing the sample. More to the point, it identifies no opinion that actually extrapolates from the sample, acknowledging that the Hospitals “cannot point to any HCA expert opinion attempting to extrapolate the sample.” (Reply, p. 4:4-6; Reiter Decl., Ex. G, § VII.C.) The harm it describes—extrapolation from 180 claims to the full population (Mot. at p. 8:18-20)—is met directly by the order in Part III.D.

The denial is without prejudice. The June 29, 2026 order sets a December 1, 2026 deadline for motions in limine and *Sargon* motions. (¶¶ 4, 8.) The County reserved its *Sargon* challenges (Mot., p. 1, fn. 1), and the Hospitals anticipate such a motion as to Dr. Schriger (Opp., p. 17, fn. 3). The admissibility and reliability of the sampling methodology are properly addressed on that schedule, after expert depositions and on a full record.

D. Dr. Palega’s Opinions Are Limited

The County does not challenge Dr. Palega’s disclosure; it seeks only an order confining his opinions to the claims he reviewed and precluding extrapolation. (Mot., p. 1, fn. 1, pp. 9:18-26, 12:6-9.) The Hospitals do not address the request, and the Court finds it well taken as a matter of trial management. (Reply, p. 4:3-6.) Dr. Palega performed no statistical analysis, offered no opinion on any claim he did not review, and did not opine that his conclusions may be projected to the balance of the claims. (Reiter Decl., Ex. F, ¶¶ 19-20; Ex. G, § VII.C.)

Nevertheless, the Court declines to enter the order as proposed. Dr. Palega reviewed 178 of the 180 sampled claims, two having been withdrawn after the sample was drawn (Reiter Decl., Ex. G, § VI.D; Reply, p. 5, fn. 1), so an order framed by a fixed number invites dispute. Furthermore, the Hospitals disclosed that Dr. Palega may offer rebuttal testimony responsive to the County’s experts (*Id.*, Ex. A at p. 5, ¶ 8), which, in light of Part III.E, he may need to do. The order is therefore:

No party may offer, and no expert may give, opinion testimony or argument extrapolating or projecting the conclusions of Dr. Palega’s review of the sampled claims

to any claim that Dr. Palega did not review, absent further order of the Court. Nothing in this order limits Dr. Palega's ability to offer testimony responsive to opinions offered by the County's experts.

E. Alternative Motion to Augment

1. Legal standard

A party may move to augment its expert witness list, sufficiently in advance of the discovery cutoff to permit the new expert's deposition within that limit. (Code Civ. Proc., § 2034.610, subds. (a)(1), (b).) Leave shall be granted if the court has taken into account the opposing party's reliance on the existing list, that party will not be prejudiced in maintaining its action or defense on the merits, and the moving party either would not in the exercise of reasonable diligence have determined to call the expert, or failed to designate through mistake, inadvertence, surprise, or excusable neglect and thereafter sought leave promptly and served the required information. (*Id.*, § 2034.620, subds. (a)-(c); Mot., pp. 10:5-11:17; Opp., p. 15:11-18.) Leave may be conditioned. (Code Civ. Proc., § 2034.610, subd. (d).)

2. Reliance and prejudice

The Hospitals do not argue prejudice; their opposition rests on subdivision (c) and on the scope of the two reports. (Opp., pp. 15:8-18:10.) The Court finds none. Both reports were served July 10, 2026; expert discovery runs to September 18, 2026; and the deadlines for witness lists, motions in limine, and the settlement conference are all still ahead. (June 29, 2026 Stipulation and Order, ¶¶ 2-6.) The Hospitals will have a full opportunity to depose both experts and, under the conditions below, to respond. Nor is there undue reliance. The Court's admonition that the continuance would not afford the County a "redo" (Forcini Decl., Ex. 12 at p. 32:8-11; Opp., pp. 5:5-7) was addressed to the fact discovery deadlines that had already passed; in the same ruling the Court held open "those pending outstanding dates for expert rebuttal reports and expert discovery." (*Id.*, p. 32:13-16; Reply, p. 9:7-10.) This motion is directed to the phase the Court preserved.

3. *Dr. Diver*

Leave is granted as to Dr. Diver. The County satisfies subdivision (c)(1). (Mot., pp. 10:1-9, 11:1-9.) Until March 5, 2026, nothing disclosed to the County indicated that any of the Hospitals' experts would design or rely upon a statistical sample. The Hospitals confirm the point: they say that when they served their disclosures "there was no reason to believe that a sample would be necessary," and that Dr. Palega was later "forced to rely on the audit sample created by Mr. Dressel." (Opp., pp. 5:15-20, 14:20-25.) A party that did not anticipate its own expert's sampling work cannot fairly charge its opponent with failing to predict it. (Reply, p. 9:14-19.)

The Hospitals object that the gravamen of the report is an attack on Dr. Fowdur, whose damages opinions were disclosed in September 2025. (Opp., pp. 17:21-18:10.) The objection has partial merit: Dr. Diver's assignment is framed as a statistical assessment of the Dressel and Palega reports "with respect to assumptions made in the damages estimation" in Dr. Fowdur's report, and Section VIII concludes that the damages assumption is "incorrect and overstated." (Reiter Decl., Ex. G, ¶ 2.) But Dr. Diver expressly "[s]ets aside" the problems he perceives in Dr. Fowdur's analysis and offers no critique of her economic model. (*Id.*, ¶ 13; Reply, p. 10:22-24.) The Court accommodates the objection by limitation rather than denial:

Dr. Diver may offer opinions concerning the design, execution, representativeness, precision, and coverage of the sample selected by Mr. Dressel; Dr. Palega's use of that sample; and whether the sample can support statistical inference or extrapolation to the population of claims in dispute, including the consequences of any deficiency in the sample for damages estimates that rest upon it. Dr. Diver may not offer opinions critiquing Dr. Fowdur's economic model, her fair market value methodology, or her damages calculation independent of the sampling issues identified above.

4. *Dr. Schriger*

Leave is granted as to Dr. Schriger, but on a narrower basis than the County requests. The County could not have anticipated the need for a physician to address the clinical

adequacy of a statistical sample it did not know would be created. Sections III through V of the report address that subject: why stabilization, authorization, medical necessity, and emergency status are patient-specific determinations requiring individualized record review, and why the variables on which Mr. Dressel stratified (i.e., aggregate billed charges and inpatient versus outpatient status) bear no clinical relationship to them. (Reiter Decl., Ex. H, §§ III-V, ¶¶ 11, 57-58, 63.) Those opinions respond directly to the undisclosed sampling work and are properly the subject of augmentation.

The balance is different in kind. In Section VI and paragraph 67, subdivisions (iv) through (vi), Dr. Schriger offers affirmative clinical opinions contradicting Dr. Palega on the sampled claims: different dates for the beginning of post-stabilization care in 59 claims; tests and treatments in 81 of the 178 claims that were not medically necessary or whose necessity is debatable; and 34 patients who should have been discharged or observed rather than admitted. (Reiter Decl., Ex. H, ¶ 67(iv)-(vi) & Appen. C.) Those are opinions on the medical necessity of the care the Hospitals provided. The County's own moving papers describe Dr. Schriger as concluding "that Dr. Palega's review is flawed and incomplete even as to the sampled records" (Mot., p. 8:6-9), and the County concedes that Dr. Palega's disclosure of that subject was adequate (Mot., p. 1, fn. 1; Reply, pp. 3:27-4:3).

The County cannot satisfy subdivision (c) as to those opinions. The September 3, 2025 disclosure stated in its opening line that Dr. Palega "has expertise in auditing medical records to determine if emergency services were medically necessary," and that he would opine on whether services "were medically necessary emergency care" and whether "claim denials by VHP were medically justified." (Reiter Decl., Ex. A at p. 5, ¶¶ 6-7.) A party intending to contest medical necessity through its own expert would have designated a physician in response. The County designated none, in September and again in December, and offers no explanation beyond its later change of counsel. (*Id.*, Ex. I, J; Opp., pp. 16:1-22.) A change in litigation strategy is not mistake, inadvertence, surprise, or excusable neglect, and the absence of a rebuttal physician cannot be attributed to a lack of notice about a disclosed subject. That the County did not know medical records would be reviewed does not alter the analysis (see

Reply, pp. 10:5-10); the foreseeable subject of rebuttal was medical necessity, not the evidentiary route to it. Leave is therefore granted subject to the following limitation:

Dr. Schriger may offer opinions concerning (a) whether stabilization, authorization, medical necessity, and emergency status can be determined without individualized review of patient medical records; (b) whether the sample designed by Mr. Dressel, and the variables on which it was stratified, can support clinical conclusions about the population of claims in dispute; and (c) what Dr. Palega's report does and does not establish, including the questions on which he expresses no opinion. Dr. Schriger may not offer affirmative clinical opinions that particular sampled patients were stabilized at times other than those identified by Dr. Palega, that particular services rendered to sampled patients were not medically necessary, or that particular patients should not have been admitted. Paragraph 67, subdivisions (iv) through (vi), and the corresponding portions of Appendix C, are excluded on that basis.

Because Dr. Schriger's critique of the sample draws in part on his review of individual records, this line will require attention at trial. The limitation runs to the opinions he may offer, not to the materials he reviewed in forming them.

5. Conditions

Leave is conditioned on the following. (Code Civ. Proc., § 2034.620, subd. (d).)

1. The County shall serve, within five court days, all information required by section 2034.260, subdivision (c) for each expert, together with all materials each considered, to the extent not already served.
2. The County shall serve conforming versions of both reports, redacted to remove the opinions excluded by this order, within ten court days.
3. The County shall produce Dr. Diver and Dr. Schriger for deposition on dates certain, to be set at the hearing.
4. The Hospitals may serve expert reports responsive to the opinions permitted by this order on or before a date to be set at the hearing, and may take any deposition made necessary by this order after the September 18, 2026 cutoff.

5. The expert discovery cutoff is extended to a date to be set at the hearing for the limited purpose of the discovery permitted by this order. All other dates in the June 29, 2026 order remain unchanged.
6. Neither expert may offer opinions at trial beyond those in the conforming reports served under paragraph 2.

Nothing in this order limits the Hospitals' right to challenge either expert by motion in limine or under Sargon on the schedule set by the June 29, 2026 order. (¶¶ 4, 7-8.)

6. *Sanctions*

Section 2034.630 requires a monetary sanction against a party that unsuccessfully opposes a motion to augment unless the court finds substantial justification. The Court so finds: the Hospitals' objections to the scope of both reports (Opp., pp. 15:19-18:10) were substantially justified and have been sustained in part. No sanctions are imposed.

IV. The July 10, 2026 Motion to Seal

The County moves to seal, in their entirety, Exhibits E through H to the Reiter Declaration: the reports of Mr. Dressel (E), Dr. Palega (F), Dr. Diver (G), and Dr. Schriger (H). (July 10, 2026 Mot. to Seal, pp. 1:11-19, 3:3-12; Marsh Decl., ¶¶ 3-4.) The motion is unopposed.

A. Legal Standard

Court records are presumed open, and a record may be sealed only on express findings that an overriding interest exists that overcomes the right of public access; that it supports sealing; that a substantial probability exists that it will be prejudiced absent sealing; that the sealing is narrowly tailored; and that no less restrictive means exist. (Cal. Rules of Court, rule 2.550(c), (d); Mot. to Seal, p. 3:14-20.) The order must state the supporting facts and seal only the documents, pages, or portions that need protection; all else remains public. (Id., rule 2.550(e)(1).) Privacy interests in medical and financial information may qualify (*In re*

Providian Credit Card Cases (2002) 96 Cal.App.4th 292, 298, fn. 3), and medical records are presumptively private (*Oiye v. Fox* (2012) 211 Cal.App.4th 1036, 1068-1070 (*Oiye*)).

A protective order designation is not itself a basis for sealing. The parties' Stipulated Confidentiality Order says so: it "creates no right to file Protected Material under seal without complying with California Rules of Court, Rules 2.550 et seq.," and requires that a supporting declaration state the basis for sealing with "sufficient particularity" to permit the rule 2.550(d) findings "without being required to review each item of Protected Material." (Stip. Protective Order [May 1, 2025], ¶¶ 21-22.) The Court notes, without deciding, that rule 2.550 does not by its terms apply to "discovery motions and records filed or lodged in connection with discovery motions or proceedings," and that the motion to strike arises under the Civil Discovery Act. (Cal. Rules of Court, rule 2.550(a)(3).) Because the parties brief the motions under rule 2.550, because the Stipulated Confidentiality Order requires compliance with it, and because the reports may be offered in connection with matters other than discovery motions, the Court applies rule 2.550(d).

B. Exhibit F and Appendix C to Exhibit H Are Sealed

Exhibit F is Dr. Palega's report: an individualized review of the medical records of 178 nonparty patients discussing, patient by patient, their presenting conditions, diagnoses, course of treatment, and timing of stabilization. (Marsh Decl., ¶ 4; Mot. to Seal, pp. 3:22-27.)

Appendix C to Dr. Schriger's report is a claim-by-claim inventory of the same patients. The Court finds: (1) those nonparty patients have an overriding privacy interest in their medical information, arising under article I, section 1 of the California Constitution and recognized in *Oiye, supra*, 211 Cal.App.4th at pages 1068 through 1070, that overcomes the right of public access; (2) the interest supports sealing; (3) a substantial probability exists that it will be prejudiced absent sealing, because public filing would disclose the medical histories of identifiable nonparties and could not be undone; (4) the sealing is narrowly tailored, because the confidential medical information is the substance of these materials rather than discrete passages within them; and (5) no less restrictive means exists, because redaction sufficient to

protect the patients would remove essentially all of the content. Exhibit F and Appendix C to Exhibit H are sealed in their entirety.

C. Exhibits E and G, and the Balance of Exhibit H

As to the remaining materials, the motion does not support the required findings. The supporting declaration states that the confidential information “is not confined to discrete portions of the Expert Reports but pervades them.” (Marsh Decl., ¶ 5.) Having reviewed the lodged materials, the Court cannot so find. Exhibit E, Mr. Dressel’s report, is eight pages of text followed by a curriculum vitae, a materials list, and exhibits. The body identifies no patient. It reports aggregate figures (13,188 claims, billed charges of approximately \$571,480,917, and a six-cell stratification table) and cites public federal audit and CMS sources. (Reiter Decl., Ex. E, ¶¶ 29-35 & fns. 12-19.) Its single reference to an individual patient, in footnote 12, already appears redacted, and the patient-level material is in Exhibit 3 to the report rather than the report itself. (*Id.* at ¶ 33.) Exhibit G is a statistical analysis of sampling objectives, precision, bias, and coverage; Exhibit H, apart from Appendix C, is largely clinical methodology, academic literature, and the categories of information required to assess stabilization, authorization, and medical necessity. (*Id.* at Ex. G, §§ IV, VI-VII; Ex. H, §§ III-V.)

Figures of that kind are in any event already public, appearing in the parties’ publicly filed briefs and this Court’s prior orders. (E.g., Mot., pp. 1:6-8, 8:18-20; Opp., p. 13:13-15.) Rule 2.550(d)(3) is not satisfied as to information already public. Nor does the record support the asserted business-confidentiality interest as to Exhibits E and F, which are the Hospitals’ reports, designated by the Hospitals. The only declaration is from counsel for the County, who is not positioned to establish the competitive sensitivity of the Hospitals’ information or the harm disclosure would cause (Marsh Decl., ¶¶ 1, 4, 7), and the Hospitals have neither joined the motion nor filed a declaration. The Court’s April 23, 2026 order granting an unopposed sealing motion does not compel a different result; that motion concerned a single spreadsheet exhibit, while this one seeks to seal four expert reports running to hundreds of pages. (Apr. 23, 2026 Order, pp. 2-4.)

The motion is denied without prejudice as to Exhibits E and G and as to Exhibit H other than Appendix C. Because denial would affect materials designated by the Hospitals, who have not been heard, the Court will not direct that they be placed in the public file at this time. Instead, on or before August 26, 2026:

1. Any party may file a renewed motion to seal as to Exhibits E and G and the body of Exhibit H, supported by a declaration from the designating party and accompanied by proposed redacted versions identifying the specific passages for which sealing is sought. (See Stip. Protective Order, ¶ 21.) The motion shall also address whether patient identifiers can be replaced with sequential pseudonyms, and whether Exhibit 3 to Mr. Dressel's report may be sealed separately from the report.
2. If no renewed motion is filed by that date, the lodged records shall be handled in accordance with rule 2.551(b)(6).

V. The August 12, 2026 Motion to Seal

On August 6, 2026, the Hospitals lodged conditionally under seal, under rule 2.551(b)(3), Exhibits 4, 5, 6, and 8 to the Forcini Declaration and portions of their opposition brief, all designated confidential by the County, stating that they did not intend to move to seal. (Aug. 6, 2026 Notice of Lodging at p. 2.) On August 12, the County filed this timely motion, seeking to seal specified portions of its Reply and of the Opposition that quote or summarize the Diver and Schriger reports. (Aug. 12, 2026 Mot. to Seal, pp. 1:11-22, 4:28-5:2; Givertz Decl., ¶¶ 3-4.)

The County has narrowed its request considerably: unlike the July 10 motion, this one identifies specific lines and is accompanied by a declaration itemizing them. (Givertz Decl., ¶ 4.) Even so, the motion does not satisfy rule 2.550(d). Based on the Court's review, the passages contain no patient identifier, no diagnosis, no course of treatment, no stabilization time, and no claims-level billed charge or reimbursement figure. They consist of the experts' descriptions of their own assignments, general propositions of methodology, and attorney argument:

- The Reply passages quote Dr. Schriger’s statement that stabilization, authorization, medical necessity, and emergency status “are inherently patient-specific inquiries that generally cannot be determined for a given patient without, at a minimum, a patient-specific, comprehensive review of the medical records,” and Dr. Diver’s descriptions of his assignment and of the derivation of Dr. Fowdur’s estimate, without figures. (Reiter Decl., Ex. H, ¶ 11; Ex. G, ¶¶ 2, 13.) These are propositions about no patient in particular, and the first appears in substance in the County’s publicly filed moving papers. (Mot., pp. 7:25-8:6.)
- The Opposition passages at page 15, lines 20-21 and page 17, lines 23-24 describe Dr. Schriger’s role and argue that he is an emergency physician rather than a statistician; page 18, lines 1-8 quote the same Diver paragraph. That is argument, not protected material.
- The Opposition passage at page 16, line 24 through page 17, line 7 quotes and discusses the County’s own motion to strike. (Opp., pp. 16:23-17:4, quoting Mot., pp. 7-8.) The County represents that it does not seek to seal that motion. (July 10, 2026 Mot. to Seal, p. 4:21-24.) A party cannot establish a substantial probability of prejudice from the public filing of material it has itself placed in the public file.

The overriding interest the County asserts—the privacy of nonparty patients (Mot. to Seal, pp. 3:22-4:3)—is substantial, and the Court has given it effect in Part IV.B by sealing the materials that actually contain patient information. It is not implicated by counsel’s characterizations of what those materials say. Narrow tailoring under rule 2.550(d)(4) and (e)(1) requires that a seal reach the protected material and stop there. Sealing argument about a sealed document, where the argument itself discloses nothing protected, extends the seal past its justification. The motion is denied, and the identified portions of the Reply and Opposition shall be placed in the public file.

The County does not seek to seal Exhibits 4, 5, 6, and 8 to the Forcini Declaration or the portions of the Opposition at page 9, lines 23 through 26, and page 10, lines 14 through 16. (Mot. to Seal, pp. 1:18-22.) Under rule 2.551(b)(3)(A)(iii), those materials shall be placed in

the public court file. Exhibits 4 and 5 are the County's supplemental interrogatory responses, substantial portions of which this Court has already quoted in its publicly filed orders.

Finally, the County lodged with its Reply excerpts of the Berryhill and Atkinson deposition transcripts as Exhibits 1 and 2 to the Supplemental Reiter Declaration. (Aug. 12, 2026 Notice of Lodging, p. 1; Suppl. Reiter Decl., ¶¶ 3-4.) Neither the motion nor the Givertz declaration requests sealing of those exhibits, and the Court does not order them sealed. They appear to have been designated by the Hospitals, who under paragraph 20(b) of the Stipulated Confidentiality Order may move to seal within ten business days after lodging; any such motion shall be filed on or before August 26, 2026. If none is filed, the exhibits shall be handled under rule 2.551(b)(6).

VI. Conclusion

1. The motion to strike Mr. Dressel's opinions is GRANTED as to Opinion No. 1 and the supporting discussion at paragraphs 19 through 28 of his report and is otherwise DENIED without prejudice to a timely motion in limine or Sargon motion.
2. The motion to limit Dr. Palega's opinions is GRANTED as modified in Part III.D.
3. The alternative motion to augment is GRANTED as to Drs. Diver and Schriger, subject to the scope limitations in Parts III.E.3 and III.E.4 and the conditions in Part III.E.5. No sanctions are imposed under section 2034.630.
4. The July 10, 2026 motion to seal is GRANTED as to Reiter Declaration Exhibit F and Appendix C to Exhibit H, and DENIED WITHOUT PREJUDICE as to Exhibits E and G and the balance of Exhibit H, on the terms in Part IV.C.
5. The August 12, 2026 motion to seal is DENIED. The materials the County does not seek to seal shall be placed in the public file under rule 2.551(b)(3)(A)(iii).

The Court will prepare the order.

Calendar Line 3

Case Name:

Case No.:

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Calendar Line 4

Case Name:

Case No.:

Calendar Line 5

Case Name: *Goodman v. Boba Guys, Inc.*
Case No.: 21CV383976

This action was filed on July 2, 2021, and N. William Metke of The Mitzel Group LLP (“Counsel”) is counsel of record for defendant The Boba Guys, Inc. (“Defendant”). Counsel has filed a motion to be relieved as counsel for Defendant under Code of Civil Procedure section 284(2). The motion is unopposed. As discussed below, the Court GRANTS the motion.

I. Legal Standard

Motions to be relieved as counsel are technical and governed by rule 3.1362 of California Rules of Court (“Rule 3.1362”). Notice and motion must be directed to the client on Judicial Council Form MC-051. No memorandum is required. (Rule 3.1362(a) & (b).) Counsel must provide a declaration on Judicial Council Form MC-052 stating “in general terms and without compromising the confidentiality of the attorney-client relationship why a motion under Code of Civil Procedure section 284(2) is brought instead of filing a consent under Code of Civil Procedure section 284(1).” (Rule 3.1362(c).)

The notice of motion and motion, the declaration, and the proposed order must be served on the client and all parties “by personal service, electronic service, or mail.” (Rule 3.1362(d)). Rule 3.1362(d) sets forth the service requirements, as follows:

If the notice is served on the client by mail under Code of Civil Procedure section 1013, it must be accompanied by a declaration stating facts showing that either:

(A) The service address is the current residence or business address of the client;
or

(B) The service address is the last known residence or business address of the client and the attorney has been unable to locate a more current address after making reasonable efforts to do so within 30 days before the filing of the motion to be relieved.

(Rule 3.1362(d).)

The proposed order relieving counsel must be prepared on the Order Granting Attorney’s Motion to Be Relieved as Counsel—Civil (form MC-053) and must be lodged with the court with the moving papers. The order must specify all hearing dates scheduled in the action or proceeding, including the date of trial, if known. If no hearing date is presently scheduled, the court may set one and